Patient 14

Report A

6527774 11 O6/2016 CT Acute Stroke

For Thrombolysis? Yes

Report:

Time 1st called: 16:20

Time plain CT available on PACS:

Time of verbal report to stroke registrar:16:25

indication: Exp aphasia and right facial droop.

CT HEAD

Haemorrhage: No.

Intravascular thrombus: No acute thrombus demonstrated on non-Contrast CT. Acute infarct: Possibly- There is some mild loss of grey-white matter differentiation within the left insula, possibly representing an acute infarct. Volume loss in the posterior insula most probably reflects a mature infarct, related to the mature infarct in the frontal operculum Other findings and comments: There is a focus of calcification projected over the left insular branch of the MCA.

CT ANGIOGRAM

Aortic arch and Subclavian arteries: Moderate atherosclerotic disease of the arch.

RCCA/ICA: Mixed atherosclerotic plaque at the bifurcation causing approximately 25% stenosis

LCCA/ICA: Mixed atherosclerotic plaque at the bifurcation causing approximately 25% stenosis.

RWA: Hypoplastic, terminating in PICA. A small calcified plaque at the origin is noted.

LVA. Dominant. Calcified palque at the V1 origin and V2 segment. Tortuous vessel with 360 degree loop in the V3 segment. Intracranial Arteries: There is a focus of calcification projected over the left insular branch MCA with reduced flow seen distally. No intracranialaneurysm.

Wall calcifications of the C3 segments are noted. A small calcified plaque is seen in the left V4 segment. Other findings and comments: A small mature infarct is noted in the left frontal operculum. Thee is generalised volume loss. Severe degenerative changes in the cervical vertebrae.

Conclusion: Possible acute infarct in the left insular lobe. No acute haemorrhage.

Report B

6527963 12/06/2016 CT head

Clinical information: At 4pm this evening please; post thrombolysis

Findings: Non contrast imaging obtained. Comparison made to stroke protocol CT dated 11/06/2016. Marginally increased delimitation of an infarct centred upon the left insular cortex, left frontal and temporal opercula is shown without convincing extension when compared to the prior examination and with no haemorrhagic transformation.

A tiny fleck of calcific density material that was previously seen within a proximal M2 branch of the left middle cerebral artery has migrated into a more distant M3 location and appears to have slightly fragmented into three miniscule specks. Extensive calcific atheroma at the origin of the left common carotid artery origin or the left carotid bifurcation/proximal ICA are the most likely sources for this suspected Calcific atheroembolus.

No other changes.